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ACCT: K509976683 LOC: K.327 1
EXAM DATE: 05/25/2026 ST: ADM IN
UNIT NO: K00395352
EXAM COMPLETE: 1820

EXAMS:
001137053 MR Lumbar wo/w Con

MR CERVICAL, THORACIC, AND LUMBAR SPINE WITHOUT THEN WITH IV CONTRAST
DATE: 5/25/2026
HISTORY: Reason for Exam? r/o mets DX: DYSARTHRIA Comment?
(K509976683)
TECHNIQUE: MR examination of the cervical, thoracic, and lumbar spine was performed without and then with intravenous contrast per the routine protocol.
COMPARISON: 5/24/2026 MRI of the brain, 5/25/2026 CT of chest abdomen and pelvis
FINDINGS:
CERVICAL SPINE:
Alignment is normal.
The craniocervical junction is normal.
There is no fracture. Marrow signal is unremarkable and without abnormal marrow replacement.
Cervical cord signal intensity is normal. Enhancing posterior fossa brain lesions similar to the recent brain MRI.
Multinodular thyroid gland. No other prevertebral soft tissue findings. There are normal flow voids in the vertebral arteries.
Mild disc bulge at C5-6. No significant spinal or foraminal compromise.
THORACIC SPINE:
Alignment is normal.
There is no fracture. Probable vertebral body hemangioma at T9 with chronic 35% compression fracture deformity. No suspicious marrow replacing lesions or suspicious enhancement identified.
Thoracic cord signal is normal.
Paravertebral soft tissues are normal.
Chronic posterior superior border of T9 without significant spinal stenosis.
Mild disc space narrowing with relative disc desiccation at T2-5. No definite significant facet hypertrophy. No significant spinal or foraminal compromise.
LUMBAR SPINE:

Alignment is normal.
Minimal chronic superior endplate compression fracture deformity at L3 and anterior superior end plate Schmorl's node. Marrow signal is unremarkable and without abnormal marrow replacement.
The conus medullaris is at the level of L1, and the distal spinal cord signal intensity is normal.
Very large uterine fibroids. Small nonenhancing renal cortical lesions most consistent with cysts. At least one small enhancing subcentimeter lesion seen at the posterior right hepatic lobe incompletely imaged.
Minimal disc space narrowing with relative disc desiccation at L2-3 where there is mild disc bulging. Minimal L3-4 and right L4-5 facet hypertrophy. No significant spinal or foraminal compromise.
No suspicious enhancement following contrast administration.

IMPRESSION:

1. No findings to indicate metastatic disease to the spine.

2. Chronic compression fracture deformities at T9 and L3.
3. Mild spondylosis. No evidence of significant spinal or foraminal compromise and no focal disc herniations.
4. Enhancing posterior fossa brain lesions similar to the recent brain MRI.
5. At least one small enhancing lesion at the posterior right hepatic lobe incompletely imaged, corresponding to an enhancing lesion seen on CT.

Signed on 5/25/2026 6:46 PM by Dipan L Patel, M.D.

** Electronically Signed by Dipan Patel MD on 05/25/2026 at 1846 **
Reported and signed by: Dipan Patel, MD

CC: Neel M Shah DO

TECHNOLOGIST: Valladolid,Mauricio
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